Browning Physical Therapy

32858 Rivas Glens Suite 451
League City, TX
Phone 713-555-2368

Itemized Medical Bill

Patient:	Sarah Perez
Date of birth:	February 10, 2002
Insurance claim:	C-1350
Date of injury:	July 3, 2024
First visit:	July 8, 2024
Last visit:	January 12, 2025
Number of visits:	29

Services

Service	Units	Charge
99203 New patient evaluation	1	$254.00
98941 Spinal manipulation	5	$530.87
97110 Therapeutic exercise	6	$1,003.31
97124 Massage therapy	4	$413.99
72040 X-ray, cervical spine	6	$5,743.89
20552 Trigger point injection	7	$3,423.41
TOTAL		$11,369.47

Recent visits

Date	Service	Charge
December 8, 2024	98941 Spinal manipulation	$91.16
December 17, 2024	20552 Trigger point injection	$545.02
December 20, 2024	20552 Trigger point injection	$484.63
December 27, 2024	20552 Trigger point injection	$526.90
January 5, 2025	97124 Massage therapy	$102.94
January 12, 2025	20552 Trigger point injection	$329.30

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Treatment History and Summary

Patient:	Sarah Perez
Treating provider:	Ruben Clark, DC
Diagnosis:	knee injury

Treatment history by month

Month and services	Visits	Charges
July 2024: chiropractic adjustment, evaluation, imaging, massage therapy	4	$1,572.39
August 2024: chiropractic adjustment, massage therapy, physical therapy	4	$564.94
September 2024: imaging, pain management, physical therapy	5	$3,263.34
October 2024: imaging, massage therapy, physical therapy	5	$1,766.17
November 2024: chiropractic adjustment, imaging, pain management	4	$1,764.83
December 2024: chiropractic adjustment, pain management	5	$2,005.56
January 2025: massage therapy, pain management	2	$432.24

Treatment summary

Patient presented following a rear-end motor vehicle collision with ongoing right knee pain, swelling, and reduced range of motion. Over the course of 29 visits, care consisted of physical therapy, activity modification, anti-inflammatory medication, and a structured home exercise program. The mechanism of injury, a rear impact with forward and rebound loading of the lower extremity, is consistent with the type of knee trauma described. Early visits were more frequent, and the frequency of visits decreased steadily as symptoms improved and strength and mobility returned. Examination findings include localized tenderness and a positive effusion sign along the medial joint line, which support the reported injury. Other ligamentous and neurovascular tests were within normal limits. The patient reports no prior injury or treatment involving this knee, and no earlier recrods were identified that would suggest a pre-existing condition. Billing records were reviewed and reflect no duplicate or repeated charges for the same services. At discharge, the patient was improving with residual mild stiffness and was released to independent exercise with follow-up as needed.